

AAC Sector Briefing

27 February 2024



RESTRICTED, NON-SENSITIVE

Programme Flow

Segment One

Agenda

Welcome & Introduction

- i. Understanding the AAC Service Model

Befriending and Buddying (Bnb)

- i. Social isolation risk factors;
- ii. Requirements for befriending and buddying;
- iii. Referral process;
- iv. Capabilities of staff and volunteers

Information and Referral to Care Services

- i. Recap on information and referral to care services

Social Connector & Community Screening (2S)

- i. Understanding Social Connector & Community Screening;
- ii. Vital signs measurement equipment;
- iii. Referral journey for seniors requiring 2S assistance

Q&A Session

Break

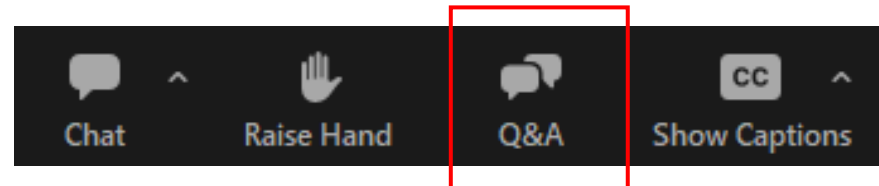
Zoom Q&A Session



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For Age Well SG

AAC 2.0



#1 Partnerships

- Door-knocking and CS completion supported by SGO.
- AACs to leverage SGO data and other community partners for outreach and profiling of seniors.

#2 Programming

- Improve structure and quality of Active Ageing Programmes at AACs.
- AACs to entice/motivate seniors out of their homes to form social circles and reduce social isolation.

#3 Resourcing

- New funding model for AACs
- Higher quantum

Background and Purpose

Background

AAC 2.0 will be progressively implemented from FY24 with a phased onboarding approach, for all eligible centres to be on AAC 2.0 by FY2026.

Intent of the updated AAC service model is to mirror, as closely as possible, the intent and requirements of AAC 2.0.

Purpose

This webinar is aimed at supporting AACs to understand the AAC service model through:



Active Ageing



Befriending and Buddying



Information and Care Referral



2S and Collaborations

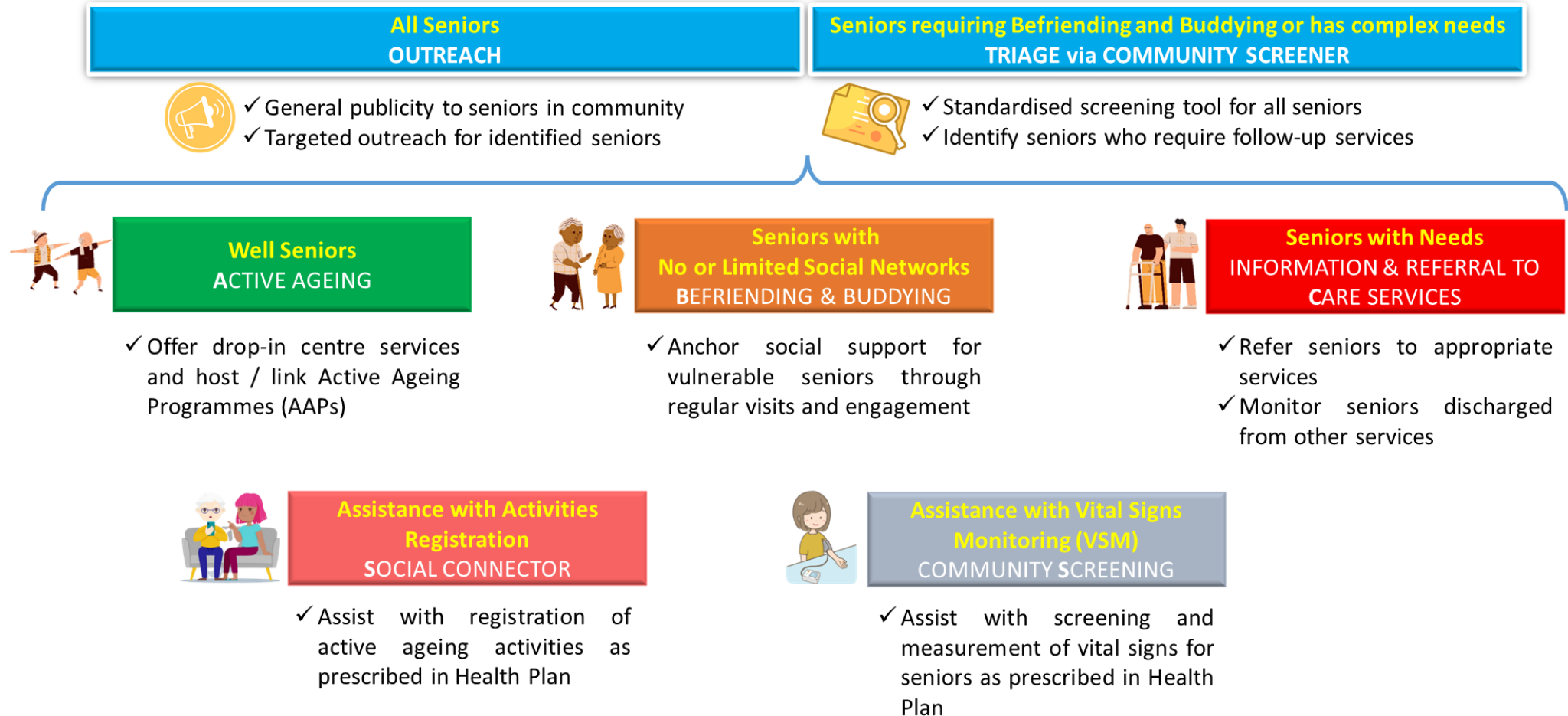


Capability Building



Funding, KPIs, and Progress Reporting

Fundamentals of AAC



Expansion of Eligibility Criteria for AACs, AACs are required to serve the following population:

- i. Singaporean/Permanent Resident seniors outside of the AAC's assigned service boundary;
- NEW** ii. **Singaporean/Permanent Resident spouses (aged below 60) of Singaporean/Permanent Resident seniors; and**
- NEW** iii. **Non-Resident spouses (regardless of age) of Singaporean seniors**

AACs need to provide Buddying and Befriending services based on Social Isolation Risk Factors

Befriending and Buddying Service

Provides emotional support to socially vulnerable seniors with 1 or more of the 4 Social Isolation Risk Factors:

1

Living alone or as good as living alone, and feels lonely

2

Limited or no social support

3

Primary caregiver of someone with high care needs

4

Limited social interactions due to physical or cognitive impairment

Community Screener (CS)

A tool used by AACs to screen seniors for the 4 risk factors and determine if they require Befriending/ Buddying.

For seniors who had been engaged by SGD, their CS results will be shared with AACs through the quarterly data sharing of SGD data.

AACs to re-administer CS if there are changes to seniors' condition, regardless of current CS's validity.

From 1 April 2024, CS validity period will extend from 1 year to 2 years.

Befriending and Buddying – Service Requirements



Befriending

Qualifying Criteria

- Singaporean or PR, aged 60 and above.
- **Min. 2 risk factors** indicated in CS/ SGO Data.
- Referral raised in IRMS.

Service Requirement

- 1x contact with senior **weekly**¹.
- At least 1x face-to-face contact with senior.
- **every 2 weeks**.

If senior is uncontactable...

- 2 unsuccessful attempts to contact senior in the **week**¹ will be deemed as fulfilment of the contact requirement.



Buddying

Qualifying Criteria

- Singaporean or PR, aged 60 and above.
- **1 risk factor** indicated in CS/ SGO Data.
- Referral raised in IRMS.

Service Requirement

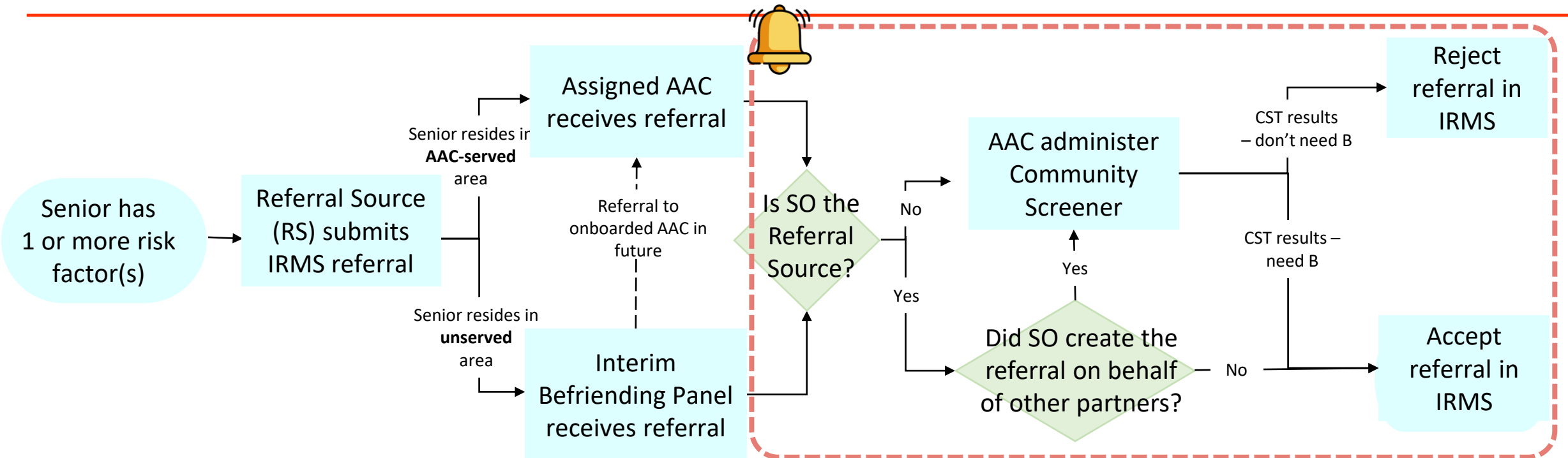
- 1x contact with senior **monthly**¹.
- At least 1x face-to-face contact with senior. **every 3 months**.

If senior is uncontactable...

- 2 unsuccessful attempts to contact senior in the **month**¹ will be deemed as fulfilment of the contact requirement.

¹AACs are required to document all attempts to contact the senior in the event of audit.

Referral Process – New referrals (w.e.f from 1 April 2024)



Is IRMS referral needed?

All new Bnb referrals must be referred via IRMS:

- For tracking purposes
- To reduce drop cases



How to check if SO created the referral on behalf of other partners?

AACs may refer to remarks box in IRMS referral to confirm and redo the CS as seniors' condition may change recently.

Befriending and Buddying – Capabilities of Staff/Volunteers

Staff and/or volunteers delivering Bnb services should be **trained** to possess the following capabilities.

Observe seniors' overall well-being

- Identify signs of self-neglect or risk of self-harm.
- Report changes or red flags (e.g., deterioration in physical health).
- Look out for home hazards.



Encourage participation in social activities

- Encourage senior to participate in activities at the AAC.
- Facilitate sessions held in the community.



Provide emotional support

- Develop supportive relationships with senior and /or their caregivers.
- Engage senior in mind-stimulating activities (e.g. games).



Administer Community Screener

Ensure that minimally 2 staff per AAC:

- Are trained to administer CST.
- Maintain CS accreditation.
(by completing 5 CS every year)



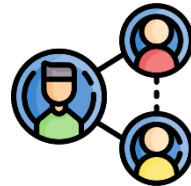
Information and Referral

- 
- AACs will serve as a key go-to point for social and health matters to seniors and their caregivers.
- AACs will be the first-stop for seniors for their social and health-related matters.
 - AACs may not render all the services as required by the seniors.

Should the required services of the seniors not be provided by the AAC, the AAC should then provide:



Information on relevant schemes, grants, and support available.



Referrals to other services (if required).



Escalation to AIC if seniors have further needs.



To Note

- Furnish all pertinent information to senior.
- Provide continued assistance to the senior until necessary support is obtained.
- Obtain explicit consent prior to making any referral.
- Track the outcome of the referral, as far as possible.

2S: Social Connector & Community Screening

Healthier SG was launched on 1 July 2023. As part of Healthier SG's initiatives, AACs shall serve as:

1. A **Social Connector** to help seniors follow through with social and lifestyle interventions as recommended by their family doctors.
2. Community nodes to provide assistance in **Community Screening** (e.g. vital signs monitoring) for seniors who have been advised by their family doctor to do so.



**Note: These services are not boundary-specific.*

The services may be rendered through referrals of seniors, or seniors who walk into AAC to request for assistance.

Should your AAC receive a referral from the RHCs/PCN HQs for seniors that are residing outside of your service boundary, this means that:

- i. The senior's current postal code of residence is not being served by any AACs; and
- ii. Your AAC is the nearest AAC to the senior's place of residence.

Your AAC are expected to provide 2S assistances to these referred seniors.

Vital Signs Measurement Equipment

AACs are required to always have a functioning vital signs measurement equipment that would be able to measure the following vital signs:

- a) Weight;
- b) Blood pressure; and
- c) Heart rate.



Weighing Scale

- AACs may purchase the general weighing scales available in the market.
- Weighing scales that are integrated with blood pressure monitor may be classified as a medical device – please reach out to Health Sciences Authority (HSA) for clarifications on your integrated equipment.



Blood Pressure Monitor

- AACs may procure general blood pressure monitors in the market.

Referral Journey of Seniors Requiring Assistance with 2S



Senior enrolls in Healthier SG and consults his/her family doctor to complete his/her health plan.



If senior requires assistance, clinic / primary care coordinator / Clusters will refer senior to AAC via email.



AAC staff to contact senior within 3 working days; AAC to update referral source on the contact outcome.



Contact Outcome	Next Action
Senior agrees to walk in to AAC	NA
Senior declines to visit AAC	AAC staff attempts to contact senior again within the next 1 month.
No response	AAC staff attempts to contact senior again within the next 2 working days.

Note: AAC does not need to contact the senior again if senior declined and explicitly expressed not to be contacted again. AAC also does not need to make a third contact attempt.



Senior makes a visit to the AAC (either walk-in or after being contacted by AAC).

AAC staff guides senior in accordance with the social and lifestyle interventions prescribed in his/her health plan:

- (i) Use mobile applications to register for active ageing activities in the community
- (ii) Use vital signs measurement devices and log measurements



Q&A Session

Moderator	Designation
Ms Liao Weifen	Deputy Director, Primary and Community Care Development Division (PCDD)
Panel Members	Designation
Mr Chew Chun-Liang	Deputy Chief, Silver Generation Division (SGD) & Primary and Community Care Development Division (PCDD)
Ms Cate Lin	Assistant Director, Primary and Community Care Development Division (PCDD)

Break Time!
See you at 10.30am

AAC Sector Briefing

27 February 2024



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Opening Remarks by Mr Chew Chun-Liang

(Deputy Chief, Silver Generation
Division & Primary and Community
Care Development Division)



Programme Flow

Segment Two

Agenda

Active Ageing

- | | |
|--|--|
| i. Active Ageing Programmes Framework (AAPF) | iv. Workflow on Submission of AAPs |
| ii. AAPF – Information on Five Domains | v. Sharing of AAPs by Partner Agencies |
| iii. AAPF – Classifications of AAPs | |

Collaborations

- i. Ground-up initiatives
- ii. Collaboration with Community Partners

Capability Building

- | | |
|--------------------|--------------------------|
| i. Data management | iii. IT Enablement |
| ii. Data sharing | iv. Volunteer management |

Funding, KPIs and Reporting

- i. Funding modality;
- ii. KPIs
- iii. Progress Report

Q&A Session

Closing Remarks

End

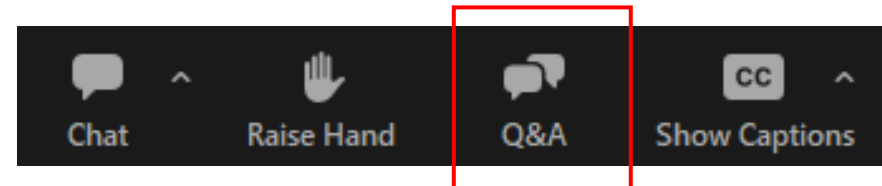
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Intent of the updated AAC service model is to mirror, as closely as possible, the intent and requirements of AAC 2.0.

Purpose

This webinar is aimed at supporting AACs to understand the updated AAC service model, with this segment of the webinar highlighting on **key changes made to Active Ageing, collaborations, capability building and funding/ KPI**



Active Ageing



Befriending and Buddying



Information and Care Referral



2S and Collaborations

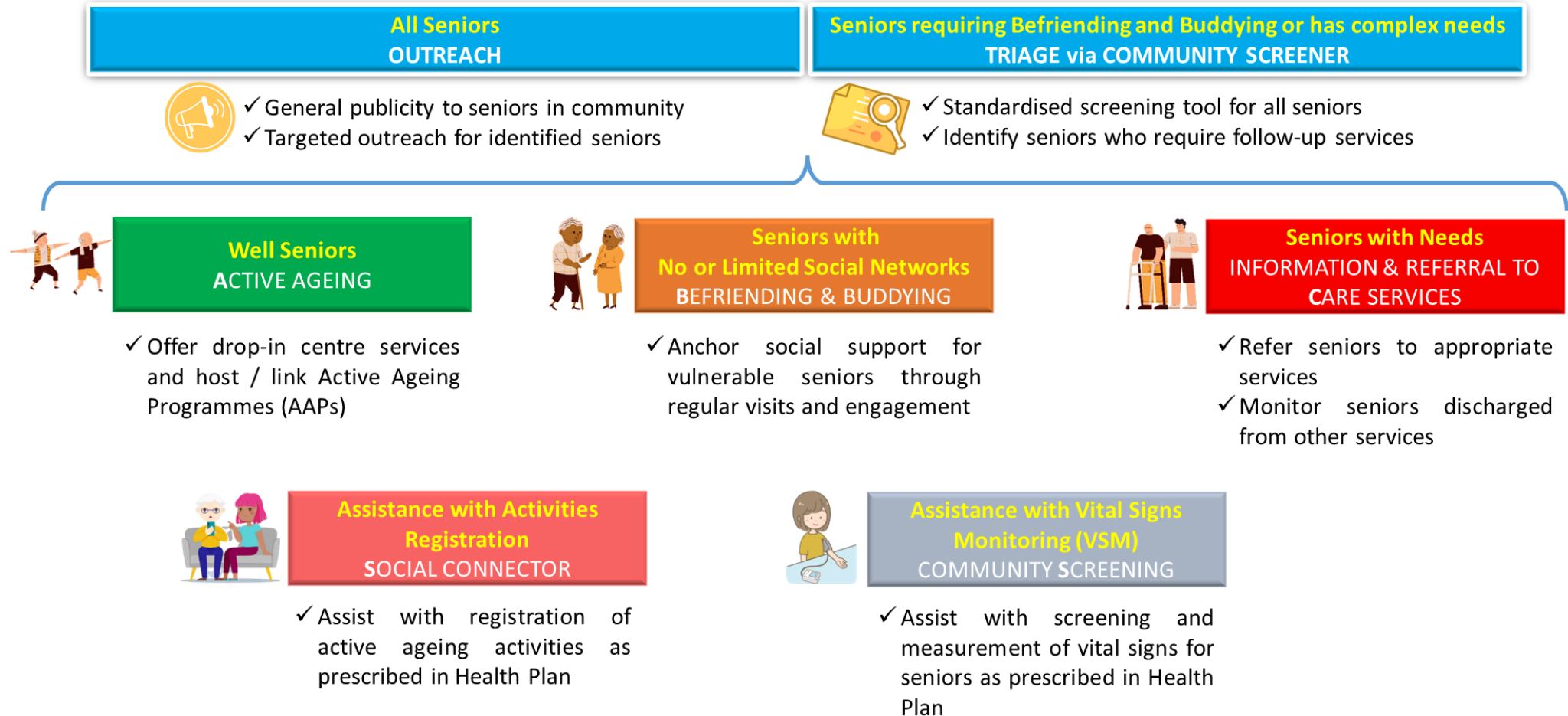


Capability Building



Funding, KPIs, and Progress Reporting

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Getting AACs ready for Active Ageing Programme Framework (AAPF)

Key Components

Only required for AAC 2.0

(1) AACs to provide programmes across 5 key domains



Physical



Cognitive



Social

Including communal meals



Learning



Volunteerism

(2) Programmes must meet baseline standards/criteria

- AAPF was developed and **endorsed by** professionals or guided by existing National Programmes / Frameworks with **clinical inputs provided by CAHOO** (Chief Allied Health Officer's Office) for the criteria and checklists.
- AACs will be provided with a **list of programmes to plug-and-play for different senior archetypes**, and
- AACs can **self-source partners** or **propose programmes** that meet the criteria based on AAPF.
- Programmes offered are classified into 2 categories:
 - (1) "Criteria-cleared" Programmes, and
 - (2) "Evidence-informed" Programmes

(3) AACs to profile seniors and recommend Programmes accordingly

Well seniors

(CFS 1 – 3)

- Attract seniors with a wider range of programmes
- Empower seniors to implement healthy behaviours

Frail seniors

(CFS 4 – 5)

- Provide relevant evidence-informed programmes, interventions and other suitable activities.

Socially isolated seniors

- Provide befriending or buddying services to encourage them to be part of community

AAPF – Information on 5 Domains

Physical 	Cognitive 	Social 	Learning 	Volunteerism 
Fitness and strengthening exercises which enhance aerobics, strength, balance and flexibility	Mind simulating activities that addresses at least 1 of 5 cognitive components: Processing speed, Memory, Attention, Executive functions and Cognitive Flexibility	Activities that can strengthen and widen seniors' social circles such as Communal meals, social programmes and social outings	Activities that support seniors in picking up new knowledge and skills such as learn about end-of-life issue; financial / health adequacy and literacy; or new skills such as learning a musical instrument	Volunteering opportunities for seniors to contribute meaningfully to the community

Communal Dining as a group activity to better foster social interactions and connectivity

Recommended Guidelines

- 1 To be conducted **at least once a week (mandatory)**
- 2 Can be held at the AAC or outside AAC (e.g. coffee shop, hawker centre, café, etc.)
- 3 15 – 20 pax with face-to-face social interactions
- 4 *Group size may be determined by seniors' profile (e.g. Socially isolated seniors may prefer smaller group size.)*
- 5 Food items (i.e. meal / snacks / desserts) can be brought from home / purchased / prepared (where possible) in the AAC
- 6 Healthy meals are recommended based on HPB's nutritional guidelines



Suggested:

Broaden AAC's outreach network by engaging Meals-on-Wheels (MOW) clients in the communal dining experience.

Note: Senior's attendance in a communal dining session can be counted towards fulfilling one count of an Active Ageing Programme (AAP) under the "Social" domain.



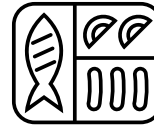
AAPF – Classification of AAPs



Physical

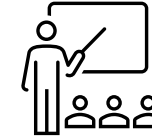


Cognitive



Social

*(incl. communal meals)



Learning



Volunteerism

*mandatory

AACs are required to offer and conduct a range of AAPs in at least
***4 out of 5 domains**, based on 2 categories

Evidence-Informed (EI) AAPs

- Active Ageing Programmes developed by / with inputs from trained professionals / experts with reference to existing literature of the proposed activity/activity modality.
- These programmes are carried out by trained personnel to ensure programme fidelity to achieve programme outcomes e.g. HAPPY Programme from HPB.

Criteria-Cleared (CC) AAPs

This refers to AAPs that meet the AAP checklist and criteria for the five domains (physical, cognitive, social, learning and volunteerism) e.g. Brisk walk programme organised by AAC.

AACs can look forward to ready AAPs offered by partner agencies as well as flexibility to conduct AAC-developed AAPs

1) AAPs Offered by Partner Agencies



AIC will disseminate the Repository of EI and CC AAPs on a quarterly basis.



AACs shortlist AAPs and approach partner agencies accordingly.



Upon agreement to conduct AAPs by partner agencies, AACs to submit collated AAP Listing[^] to [PCDD centredev@aic.sg](mailto:PCDD_centredev@aic.sg).

[^]AAP Listing Quarterly Submission Deadlines for 2024

1 st	28 June 2024
2 nd	30 September 2024
Final	30 December 2024

2) AAC's Own AAPs



Refer to AAC's existing AAP Schedule.



- Assess existing AAPs against AAPF's domain checklists
- Classify AAPs into 5 domains and EI or CC AAPs accordingly.
- Provide a brief write-up programme



Submit completed checklists for each AAP and AAC's collated AAP Listing[^] to

[PCDD centredev@aic.sg](mailto:PCDD_centredev@aic.sg).

Note that only seniors' participation in submitted AAPs can count to the workload KPI by end-FY.

Note: Approval from AIC is not required and AACs may continue running AAPs after submission. AIC will provide an acknowledgement of AACs' submission.



AACs to safekeep submitted documents for documentation and audit purposes.



AIC GRIT may observe that AACs are conducting AAPs in accordance to AAP framework requirements as necessary.

To support AACs with ready AAP programmes for Day 1, AIC is working with partner agencies, interested private companies or NGOs to bring more variety in AA programming to AACs under the 5 domains.

All activities should be made inclusive with provisions for frail seniors to participate. It may sometimes be challenging to provide inclusive physical / cognitive activities which can cater to the needs of the frail while remaining engaging for the robust.

If it is not possible, AACs should recommend frail seniors to attend targeted frail programmes (e.g. HAPPY) and other physical/cognitive activities which have been curated especially for them (e.g. Chair Zumba) as marked out in the list below. Listings are not exhaustive.



Physical



Health Promotion Board (HPB)

- ✓ Move It Feel Strong  (seated option (Chair Zumba) available for frail seniors)
- ✓ Steady Lah 
- ✓ Rolling Good Times

SportSG

- ✓ Adaptive Sports 

AACs

- ✓ Yoga
- ✓ Chair yoga
- ✓ Chair zumba
- ✓ Aerobics
- ✓ Qigong
- ✓ Zumba
- ✓ Brisk walking



Cognitive



 Targeted Frailty Programme

HPB

- ✓ HAPPY 

SportSG

- ✓ Square Stepping

NParks

- ✓ Therapeutic Horticulture Programmes at Therapeutic Gardens

Community Resource, Engagement and Support Team (CREST)

- ✓ Structured thematic activities through cognitive stimulating activities (CSA)

AACs

- ✓ Games
- ✓ Puzzles
- ✓ Meditation / mindfulness-related activities

AA programming for AACs under 5 Domains (cont'd)



Social



National Heritage Board (NHB)

- ✓ Guided Tours to museums in Singapore

Residents Committees

- ✓ Festival Celebrations

AACs

- ✓ Communal meals
- ✓ Rummy-O
- ✓ Bingo
- ✓ Mahjong
- ✓ Karaoke
- ✓ Board Games
- ✓ Inter-generational activities
- ✓ Resources developed by AIC (e.g. activity toolkits)



Learning



HPB

- ✓ Balik Kampung
- ✓ Seniors Health Curriculum

Council for Third Age (C3A)

- ✓ National Silver Academy Courses

Infocomm Media Development Authority (IMDA)

- ✓ Digital Initiatives

National Library Board (NLB)

- ✓ Time of Your Life programme

AACs

- ✓ Language classes
- ✓ Chinese Calligraphy
- ✓ Arts & Crafts
- ✓ Talks or workshops on Wills/ Lasting Power of Attorney (LPA)/ Investing Central Provident Fund (CPF), Advanced Care Planning (ACP), etc



Volunteerism



AIC

- ✓ Silver Generation Ambassadors

SportSG

- ✓ Team Nila

AACs

- ✓ Self-organised activities for seniors to volunteer in (e.g. meal delivery, goodie bag packing, activity facilitation).

AACs to further increase engagement of seniors with a greater diversity of programming in the community by tapping on Ground-Up Initiatives (GUI)

1. GUI is a new **yearly opt-in** funding stream for AACs (~\$45k per FY). This amount will be disbursed upfront as part of programme funding, AACs are given flexibility to utilise funding as long as GUI KPIs are met.

Objectives:

- a) Provide a greater diversity and vibrancy to programming in the community by leveraging ground energy and resources (e.g. volunteers).
- b) AACs as anchors in the region; GUIs to augment AAC in serving localised social/health needs of seniors.

What qualifies as GUI that AACs can work with/channel funding to:

1. Proposed activity/programme should serve seniors in partner AACs' assigned service boundary and bring seniors out of their house for some form of in-person activity and not solely providing space/venue for AAC usage only.
2. External organisations or individuals/interest groups that are not from AAC/AACs' parent organisation¹, clusters or other government agencies (e.g. HPB, NParks).
3. Proposed activity/programme is not concurrently receiving funding from another government agency.

¹ Parent organisation: direct organisation that oversees the AAC operations

GUI Administration

1. AACs will be held to following GUI KPIs:

KPI 1: Total number of sessions delivered by **all** the GUI partners under the AAC, in the FY

- Target = 24 sessions

KPI 2: Total number of unique seniors to be served (i.e., brought out for activities) by **all** the GUI partners under the AAC, in the FY

- Target = 160 unique seniors

2. GUI indicators to be reported in AACs' progress report:

- GUI partner and programme details
- Number of sessions organised and unique seniors served
- Participant feedback

3. AAC will be subject to clawback of GUI funding:

- 100% if AACs did not have any GUI partners and activities, or
- 50% if AACs have GUI partners/activities but did not meet all GUI-related KPIs.

4. The same senior reported for GUI KPI can also be counted towards AACs' KPI 1 if the GUI activity is:

- Co-organised by the GUI partner and AAC
- Listed in Repository of Evidence-Informed and Criteria Cleared AAPs

Collaborations with RHCs through CHP Services

To tighten the nexus between health and community care, AACs, to function as key community nodes to provide seniors with access to basic healthcare service.



Aim

MOH and AIC will work with Regional Health Systems (RHCs) to site all Community Health Posts (CHP) in all^{1, 2} AACs.



Frequency

Minimally once a month, for RHCs to increase frequency based on demand.



Services Rendered

Across the 3 clusters, CHPs are currently offering similar services such as geriatric assessment and chronic disease management, as well as other forms of support and intervention in lifestyle coaching, and case management and coordination.

MOH/AIC are working towards establishing a baseline suite of services across the RHCs to improve access and allow consistent experience at AACs. MOH/AIC will confirm these services by end of 2024.

¹To take into account existing AAC in-house capabilities and space constraints. Should AACs have insufficient space, Clusters and AACs could explore siting CHP at a nearby location, i.e. RN spaces.

²Clusters to work to site CHP in new AACs (i.e. newly built, recently onboarded) within 3 months.

Collaborations with RSHes through CHP – What is Required of AACs?

The role of the AACs in collaborations with RSHes through CHP is to:



Provision of centre space for siting of CHP

- An area of about 20m² for the siting¹ of CHP.
- A private room (if possible) or a common space with tables and chairs.
- To have a minimum frequency of CHP service delivered at least once a month.



Inform seniors of the availability of CHP services

- To leverage existing publicity and/or communication channels with seniors.
- For example, posters and WhatsApp groups.



Monitor seniors who may require closer watch in terms of their health

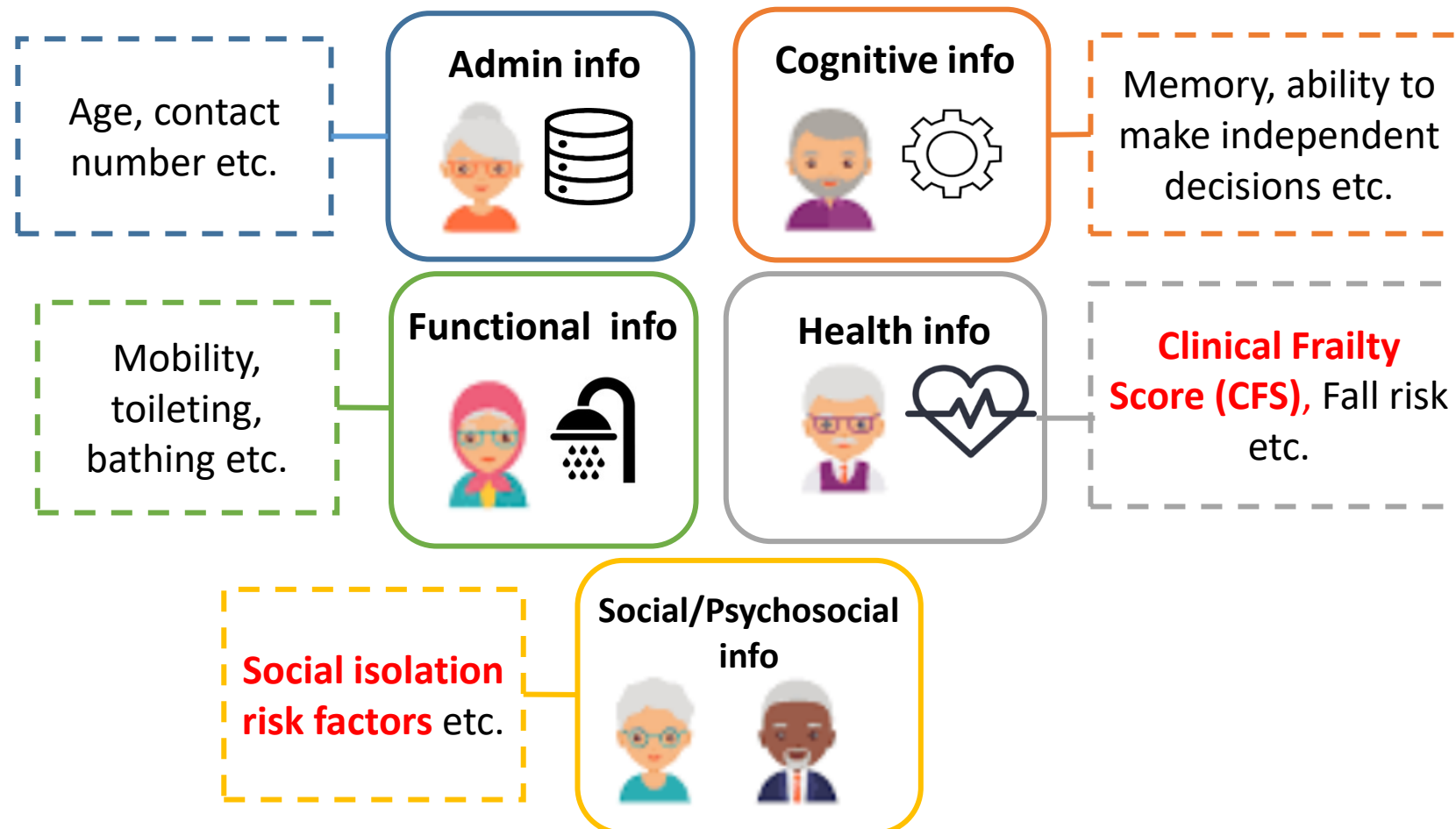
- CHP may identify some seniors that are seen in the AAC.
- AACs may mutually agree with the CHP on opportunistic monitoring of the senior (e.g. when senior participates in the active ageing programmes).

¹Should the AAC be unable to accommodate the CHP due to existing constraints, the AAC and the RHS may look to direct seniors to the nearest CHP. AACs are also encouraged to reach out to AIC to discuss on matters relating to centre space.

SGO data as key enabler for service delivery

Key areas of data provided to AACs

Data shared with AACs are responses recorded of seniors who have been successfully engaged by AIC(SGO) during Preventive Health Visits (PHV) and provided consent.



SGO data as key enabler for service delivery

What to do with the data

- With the data, AACs can better understand the seniors assigned in the service population. For example:
 - Clinical Frailty Score:** AACs plan out AAPs suitable for robust/frail seniors.
 - Number of social isolation risk factors:** AACs provide befriending/buddying services
- AACs will receive the data **quarterly** via **EasiShare**.
- The recommended follow-up actions for each senior has been specified in the data shared with AACs:

H	I	J	K	L	M	N	O	P	Q	R	S	T	U	V	W	X	Y
Gender	Ethnicity	Language spoken/understood	Postal Code	Block	Street Name	Floor	Unit Number	Working Status	Senior Contact Number	Contact Number of Next of Kin	Title of Next of Kin	Name of Next of Kin	Preferred day to call/visit	Preferred time to call/visit	Number of Risk Factor(s)	AAP Recommendation	Social Support Recommendation
Female	Chinese	Mandarin	123456	1	Chiral Hill s	1	11	Not Working		91112233	Mr.	HO	Any	Any	0	AAP (Frail)	NA
Female	Chinese	Mandarin	123456	1	Chiral Hill s	2	12	Not Working	91112233				Any	Any	0	AAP (Frail)	NA
Female	Indian	English	123456	1	Chiral Hill s	7	17	Retired	91112233				Any	Any	1	AAP (Robust)	Buddying
Male	Chinese	English~Mala	123456	1	Chiral Hill s	3	13	Not Working		91112233	Ms.	HO	Any	Any	0	AAP (Frail)	NA
Male	Chinese	English~Mala	123456	1	Chiral Hill s	4	14	Retired		91112233	Ms.	HO	Any	Any	0	AAP (Robust)	NA

Good practices in management and safeguarding of data

When using senior personal identifiable information, take steps to manage and safeguard the data with the following:



Always **obtain consent** from seniors **when collecting their personal data**

- Get a **signed consent** from the seniors when they register to be a member of your AAC. Signed consent confers seniors to:
 - Receive your centre's services;
 - Have referrals raised for other care services if necessary; and
 - Have their data used for programme evaluation and research purposes



Comply with PDPA requirements

- Have an Organisation-level **data protection policy** that cover PDPA obligations
- **Appoint a Data Protection Officer** and make his/her contact information readily available to the public
- **Publish information** on your **data protection policies**, practices and complaint-handling process



Password protect sensitive info

Password-protect sensitive data when sending via email

- Share the password to access the file with the recipient separately

Check the intended recipient of the email before sending

- Explore other avenues to share the data (e.g. transfer via external drive or removable disk)

Qualification of AAC IT Vendors and Systems

AIC adopted a 2-stage approach to develop a list of qualified IT vendors that offer AAC IT Systems based on:

- i. a set of pre-defined functional, technical and cybersecurity requirements;
- ii. vendor's capability and indicative pricing; and
- iii. vendor's commitment to keep up with new, evolving mandatory requirements.

Stage 1 - AAC IT System

- Interim system until whitelisting is ready
- Able to meet AAC needs and reporting requirements and has the minimum AAC2.0 capability
- The qualified systems will be subject to re-evaluation under Stage 2

Stage 2 - AAC2.0 IT System ("Whitelisted")

- Mandate through contractual lever
- Able to meet prevailing AAC2.0 needs, integration and reporting requirements at the point of qualification
- The qualification will be via whitelisting exercise, which will be open to vendors serving the community care sector
- Target announcement in Q3 FY2024

5 Qualified AAC IT systems for AACs to onboard to meet AAC service delivery requirements

**Active Ageing Wellness
Management System
by**



**CaritaHub AAC
by**



**GatherCare
by**



**IngoTPCC
by**



CARES



IT grant will be available for adoption of qualified AAC IT system.

- For adoption of qualified AAC IT system, AAC will be supported up to a revised IT grant cap of \$33K, sized based on average pricing across 5 qualified systems.
- Depending on the system AACs select, the norm costs of the system will be applied during the proposal evaluation process.
- Next IT grant call will be open between **1st March – 31st April 2024**. Subsequently, it will be quarterly from July 2024 onwards.
- For more clarifications on AAC IT grant, please contact your Point-of-Contact for AAC-related matters, or you may write in to AAC.IT.Grant@aic.sg

For GovTech, grant will only be available after the MOU is signed between AIC and GovTech

Engagement with Qualified Vendors

1. ENGAGE VENDOR

AACs are to engage the qualified vendors directly for the system offerings

TIP: Request for system demo

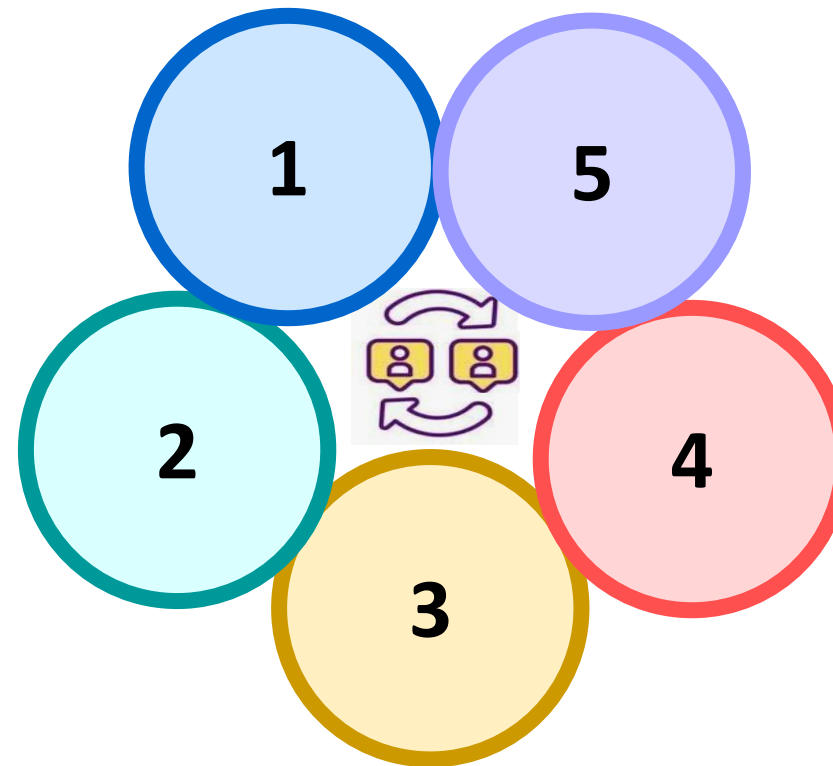
2. ASSESS SYSTEM AND VENDOR

Even though AIC has assessed the qualified systems, AACs are to carry out due diligence and make their own assessment

3. FINALISE SERVICE SPECIFICATION AND OBTAIN QUOTATION

AACs are to clearly define the service requirements and obtain a detailed quotation from the vendor

TIP: Use the Participation Agreement as reference



5. CONTRACT VENDOR

AAC can proceed to officiate the contract with the vendor upon receiving the grant approval

TIP: AACs are to adhere to their internal procurement policies and guidelines. The qualified list of systems is not a waiver of competition by AIC.

4. APPLY GRANT

With the finalised service specification and vendor's quotation, AACs can submit their AAC IT Grant application during the grant call

TIP: Vendor's quotation should be as per the costing template found at Participation Agreement under Schedule '3 - Fees'

New Volunteer Programme for AACs

New Volunteer Programme to Supplement Volunteers to AAC

1. AIC will launch a new national volunteer programme in Apr 2024 to support AACs to recruit and train additional volunteers to supplement their existing volunteer pool.
2. This national volunteer programme will also promote senior volunteerism, encouraging seniors to stay active through serving other seniors within their community.
3. Upon completion of training, AIC will then work with AACs to match these volunteers islandwide.
4. AIC seeks to gather interests from AACs to be part of the volunteer programme.
5. We will invite providers and AACs' centre and volunteer manager to attend a Webinar*, where more details of this new volunteer programme will be shared.



i. AAC Funding - Overview

AAC Programme Funding



Engagement Funding



Befriending/Buddying Funding



Start-Up Funding for Outreach



2S Funding



[Opt-in] Ground-Up Initiatives
-shared in earlier segment

New

Other Funding Streams (if eligible)



Rental Subvention



Publicity Funding
(update of AAC signages)



IT Grant

i. Funding Modality – Engagement Funding in FY2024

A Engagement Funding

For newly onboarded AACs in FY2024: Engagement funding will be based on assigned senior population.

For existing AACs: Engagement funding in **FY2024** will be determined based on the higher of the following:

a. assigned senior population

No of Assigned Seniors	Funding per FY
1,000 – 2,000	\$235,000
2,001 – 3,000	\$261,000
>3,000	\$287,000

OR

b. no. of engaged* seniors in FY23

Tier	No. of Engaged	Funding per FY
1	1 – 300	\$193,000
2	301 – 500	\$235,000
3	501 – 750	\$287,000
4	751 – 1,000	\$340,000
5	1,001 – 1,250	\$392,000
6	>1,250	\$445,000

From FY2024, engaged seniors will be redefined as seniors who have attended at least 2 in-person AAPs under one of the 5 domains.

**Unique seniors who have either:*

- (i) completed CS + have at least one (1) other face-to-face touchpoint with the AAC **OR**
- (ii) attended 2 in-person AAPs in FY2023 [capped at +1 tier from current FY23 funding]

* The previous definition i.e. Community Screener (CST) administered + one (1) other face-to-face interaction will cease from FY2024.

i. Funding Modality – Engagement Funding in FY2025

A Engagement Funding

Engagement funding in **FY2025** will be based on the tier received in FY2024, with the opportunity to move up to Tier 3:

a. assigned senior population

No of Assigned Seniors	Funding per FY
1,000 – 2,000	\$235,000
2,001 – 3,000	\$261,000
>3,000	\$287,000

b. no. of engaged seniors

Tier	No. of Engaged	Funding per FY
2	301 – 500	\$235,000
3	501 – 750	\$287,000
4	751 – 1,000	\$340,000
5	1,001 – 1,250	\$392,000
6	>1,250	\$445,000

If achieved Tier 3 – 6 for FY2024: Funding remains the same as FY24

If achieved below Tier 3 for FY2024: Centre has the opportunity to move up to Tier 3 if at least 501 seniors attend 2 in-person AAPs in FY24

i. Funding Modality – Befriending and Buddying (Bnb) Funding

B Bnb Funding

Table 1: Funding based on Bnb tiers

Tier	No. of Clients receiving Bnb	Funding per FY
1	1 – 100	\$52,000
2	101 – 150	\$78,000
3	151 – 200	\$104,000
4	201 – 250	\$131,000
5	251 – 300	\$157,000
...11 [#]	>550	\$314,000

[#] Total funding for Bnb tiers is capped at Tier 11.

- Workload achieved this FY determines the funding tier for next FY.
- Service is offered based on the outcome of the Community Screener administered by AAC or from SGO data.
- Service requirements for Befriending / Buddying must be met to be considered eligible workload:
 - **Befriending** – Weekly contact with 1x face-to-face contact with senior every 2 weeks
 - **Buddying** – Monthly contact with 1x face-to-face contact with senior every 3 months

i. Funding Modality – Start-up Funding

C Start-up Funding

Funding support for AACs to outreach to 50% of assigned population by FY2025. For FY24 onboarded AACs, the target is 35% by end of FY2025.

Funding is fixed by assigned population and with eligibility criteria to receive for Year 2 & 3 of operations.

- 1st year given
- 2nd & 3rd year:
 - At least $\geq 5\%$ increase in engaged and/or outreached seniors as compared to previous FY **OR**
 - At least 50% of assigned population engaged and/or outreached in the FY.

Engaged senior*

- 2x in-person active ageing programmes attended.

Outreached senior

- 2x interactions (face-to-face / virtual) within the FY.

* The previous definition of engagement (i.e. Community Screener administered + one (1) other face-to-face interaction) will cease from FY2024.

i. Funding Modality – Start-up Funding (Example)

C Start-up Funding

AAC Lily has onboarded in FY2023 and serves a population of 1,247 seniors.

In FY2023, the AAC had engaged a total of 317 seniors and outreached to a total of 279 seniors. Total: 596.

In FY2024, the AAC had engaged a total of 500 seniors and outreached to a total of 150 seniors. Total: 650.

	FY2022	FY2023	FY2024	FY2025
Engaged + Outreached seniors	0 (N/A)	596 (48%)	650 (52%)	
Receive Start-up Funding?		Given as default	48% - 0% =>5%	Met >50% of assigned pop in FY24
				

i. Funding Modality – Start-up Funding

C Start-up Funding

Table 1: Funding based on assigned population

Tier	Assigned Population	FY2023/24 Onboarded AACs	FY2021/22 Onboarded AACs
1	1 – 300	N.A.	N.A.
2	301 – 500	N.A.	N.A.
3	501 – 750	N.A.	N.A.
4	751 – 1,000	\$34,000	\$27,200
5	1,001 – 1,250	\$39,200	\$31,360
6	1,251 – 1,500	\$44,500	\$35,600
7	1,501 – 1,750	\$49,700	\$39,760
8	1,751 – 2,000	\$54,900	\$43,920
9	2,001 – 2,250	\$60,100	\$48,080
10	2,251 – 2,500	\$65,300	\$52,240
11	2,501 – 2,750	\$70,500	\$56,400
12	2,751 – 3,000	\$75,700	\$60,560
13	3,001 – 3,250	\$80,900	\$64,720
14	3,251 – 3,500	\$86,100	\$68,880
15	3,501 – 3,750	\$91,300	\$73,040
16	3,751 – 4,000	\$96,500	\$77,200
17	4,001 – 4,250	\$101,700	\$81,360
18	4,251 – 4,500	\$106,900	\$85,520

i. Funding Modality – 2S Funding

D 2S Funding



The 2S (“Social Connector for Social and Lifestyle interventions” and “Community Screening”) funding is provided to all AACs, in view of their key role in supporting Healthier SG.

AACs are required to provide 2S assistance to all seniors regardless of service boundary.



The 2S funding is provided as follows:

2S Funding Per FY	
FY24	FY25
\$72,120	\$74,284

AACs may use the funding flexibly (e.g. hiring of staff, procurement of vital signs monitoring equipment).

i. Funding Modality – Rental Subvention & Publicity

Rental Subvention

E Rental Subvention



Rental subvention – AACs will receive yearly subvention on a reimbursement basis at the end of FY. AAC is required to submit tenancy agreement and the relevant information for AIC's verification.

Publicity

F Publicity



One-time Publicity Funding – Existing centres onboarding the AAC service (i.e. SAC Rental or centres on Transition Grant) will receive \$6,000 after the installation of the AAC/AAC (Care) logo and/or update of their public-facing signages in line with AIC's checks and requirements. The savings can be used for other publicity materials in relation to the naming of the AACs.

AACs submit "Logo Signage Application Form" to obtain AIC's in-principal approval



AIC approves the signage artwork. After AACs installed the logo, they submit a picture of the actual signage to AIC



AICs approves \$6,000 for funding of logo/signage to be claimed in the following claim cycle.



i. AAC Funding - Summary



Engagement funding in FY24 (Quarterly)

- **Existing AACs:** Tiered funding based on revised criteria determined by whichever is higher:
 - a. Assigned senior population OR
 - b. Last achieved tier based on FY23 Engagement workload*

Completed CST with 1 F2F touchpoint
+
Attended 2x in-person AAPs (capped at +1 from FY23 funding tier if included)
- **New AACs in FY24:** Tier by assigned population.
- Min. funding of \$235,000



Befriending/Buddying funding (Quarterly)

- Tiered funding based on total Befriending /Buddying workload achieved in previous FY
- AACs onboarded in FY24 will receive Tier 1 of \$52,000



2S funding (Quarterly)

- Fixed rates by FY for all AACs. FY24: \$72,120.



Start-Up funding (Yearly)

- **Funding fixed by assigned population with eligibility criteria for Year 2 and 3.**
- Default Year 1 funding; Year 2 & Year 3 Start-Up funding will be provided if:
 - Overall % of engaged & outreached increases by at least $\geq 5\%$ from previous year OR meets at least 50% of assigned population
- E.g. \$54,900 for population of 2,000



Rental Subvention (Yearly)

- Reimbursement by end-FY, for Jan – Dec period



Publicity (One-time)

- \$6,000 for every AAC/ AAC (Care)*

* RFP sites are not eligible for publicity funding



[New] Ground-Up Initiatives (Quarterly)

- Optional at ~\$45,000 per FY



IT Grant (milestone payment)

- Quarterly grant call application

* The previous definition i.e. Community Screener (CST) administered + one (1) other face-to-face interaction will cease from FY2024.

ii. KPIs

1	Revised	Targets (By end-FY2025)
	Percentage of seniors (unique) in the assigned population who attend at least two AAPs in-person, offered or recommended by the AAC, in the FY.	≤Tier 3: 15% Tier 4: 20% Tier 5: 25% Tier 6: 30%
	Removed related existing KPI	
	Percentage of engaged seniors who have participated in at least one in-person programme offered or recommended by the AAC.	80%
	Meet at least Engagement Tier 3 for centres that started off at Tier 1; or Tier 4 for centres that started off at Tier 3.	Engagement Tier 3 or Tier 3 (depending on starting tier in FY23)
2	Revised	
	Percentage of seniors in the assigned population having at least 2 face-to-face or virtual touchpoints with the AACs.	50% (Prorated at 35% & 20% if in operations for 2 years & 1 year respectively)
	Removed related existing KPI	
	Percentage of assigned senior population successfully engaged or outreached	50% by the third year of operations

ii. KPIs

3	Revised	Targets (By end-FY2025)
	Percentage of seniors in the assigned population, who were surveyed¹ and reported that they are satisfied with the active ageing and socio-recreational programmes offered or recommended by the AAC/AAC (Care).	85% by FY2025
	Removed related existing KPI Percentage of engaged seniors surveyed who reported satisfaction with the active ageing and socio-recreational programmes offered or recommended by the AAC.	85%
4	New	
	Organise active ageing programmes in the respective active ageing domains (social, cognitive, physical, learning, volunteerism) Communal meals, under social domain, being mandatory.	Activities available in at least 4 out of 5 domains. Communal meals (once a week), under social domain, being mandatory

ii. KPIs

5 Existing	Targets (By end-FY2025)
Percentage of seniors receiving Befriending/Buddying service who reported feeling well supported.	85%
6 Existing Percentage of seniors receiving Befriending/Buddying service who have participated in at least one active ageing or socio recreational programme per month.	30%
7 Existing Percentage of seniors who approached the AAC for assistance (Information and Referral, Social Connector, and/or Community Screening) who reported satisfaction with assistance rendered.	85%

iii. Monitoring Indicators (MIs)

S/N	Monitoring Indicators
1	Average daily attendance <u>Numerator</u>: Number of seniors who participated in the activities held at the AAC or conducted by the AAC in the Financial Year <u>Denominator</u>: Number of days that the AAC was open in the Financial Year
2	[Revised] Percentage of seniors who are volunteers at the AAC (including those who are known to volunteer at other organisations) <u>Numerator</u>: Number of seniors who are volunteers (a) at the AAC or (b) is referred to external volunteering opportunities <u>Denominator</u>: Number of seniors (all age groups) at the AAC
3	[Revised] Number of seniors who visited the AAC for 2S for assistance to follow through with their Health Plan as advised by their physician. a. Number of seniors (non-unique) who received social connector services b. Number of seniors (non-unique) who received community screening services
4	[New] Number and type(s) of partnerships formed between AACs with Clusters and/or other community partners.
5	[New] Number of sessions available for each of the Active Ageing Programmes domains

iv. Progress Report

AACs are required to submit a Progress Report every 6 months.

- **October** – Mid-FY Progress Report
 - Reporting Period: Mid-FY24 (Apr 2024- Sep 2024), Mid-FY25 (Apr 2025 – Sep 2025)
 - Allows AIC to understand the current progress of your AAC
 - Render necessary support (if needed) to assist your AAC in achieving the stipulated targets
- **April** – Full-FY Progress Report
 - Reporting Period: FY24 (Apr 2024 – Mar 2025), FY25 (Apr 2025 – Mar 2026)

The Progress Report template is built-in with validation and description of data fields, and released with a guide on instructions.

For more clarifications on the data fields in the Progress Report template, please contact your Point-of-Contact for AAC-related matters, or you may write in to PCDD_Centredev@aic.sg



Q&A Session

Moderator	Designation
Ms Liao Weifen	Deputy Director, Primary and Community Care Development Division (PCDD)
Panel Members	Designation
Mr Chew Chun-Liang	Deputy Chief, Silver Generation Division (SGD) & Primary and Community Care Development Division (PCDD)
Mr Mohamed Farouk	Director, IT, Innovation & Digitalisation Division (I2D2)
Ms Cate Lin	Assistant Director, Primary and Community Care Development Division (PCDD)
Ms Keng Yik Hong	Senior Manager, Primary and Community Care Development Division (PCDD)

Thank you for attending the briefing!



- Post-webinar materials (i.e. webinar slides & recording, AAC Ops Guide, FAQ) will be disseminated at a later date.
- For any further queries, please contact the AAC Team (PCDD_centredev@aic.sg) or your GRIT POC.

Thank you.

The Heart of Care

RESTRICTED, NON-SENSITIVE



Thank you.

The Heart of Care

RESTRICTED, NON-SENSITIVE

